

You've already proven the demand is there. Here's how we make it repeatable.

The Schoeman Growth Partnership — social authority, paid acquisition, and a smarter path from attention to patient.

You're not starting from zero.

~300 → ~1,500

followers · peak week ~23,000 views

Earlier this summer, your clinic's Instagram grew from around 300 followers to roughly 1,500, with a peak week of about 23,000 views. Then visibility fell away.

We're not going to call that “going viral”, because we don't yet know what caused it. But it answers the hardest question in marketing: the right people already respond to Dr Gina. Our job is to understand what created that response, and build a repeatable way to create it, rather than hoping it happens again.

This didn't come from a template. I studied your digital presence.

Before recommending anything, here's exactly what I reviewed, and what I didn't.

Instagram — analytics

Both accounts, historical performance, follower trends, views, the June breakout, and the role each account plays.

Facebook — analytics

Both pages, and the reality of the organic audiences.

LinkedIn — content

Your posts, topics, openings, formatting and link placement. The content, not the analytics — I don't have LinkedIn's numbers, so nothing here is a LinkedIn performance claim.

How it connects

The parts of your patient journey and communication set-up that matter for where qualified interest should go next.

I'd rather be precise about what I know than impressive about what I don't.

Five things worth your attention.

1 The market has already responded to you. The June growth on your clinic account shows the appetite is real.

2 Your authority is spread across two accounts (Dr Gina ~2,350 followers; The Schoeman Clinic ~1,515). A chance to give each a clear job, rather than two calendars pulling against each other.

3 LinkedIn has the expertise; the delivery can lift it. Credible, knowledgeable and consistent, with room in the openings, formatting, link placement and reuse. (Content review.)

4 Facebook isn't worth organic effort right now (very small audiences) — separate from Meta as a paid platform, a different tool for a different job.

5 Right now, attention has nowhere useful to go. The single biggest opportunity — and we'll come to it.

There's plenty we can improve from day one.

Before a single pound of ad spend, the audit already points to changes we make in week one.

Immediate wins

- Give each Instagram account a clear role: **Dr Gina = authority, The Schoeman Clinic = trust & conversion** (a recommendation, not two disconnected calendars)
- Analyse the June breakout and rebuild on what worked
- Stronger openings and link placement on LinkedIn; repackage your expertise as native content
- Clear content pillars, hooks and calls-to-action
- Smarter reuse of your strongest topics and existing video
- Deprioritise organic Facebook; keep Meta for paid

One bigger opportunity

Your content earns attention. Paid puts proven messages in front of more of the right people. An **educational assessment** turns that attention into identifiable interest. And your existing follow-up turns interest into a patient journey.

Right now, most of that attention ends at a like. That's the opportunity — and the rest of this shows how we build it.

Content shouldn't end at engagement.

Today, someone might watch Gina, save a post, visit the profile, think “this sounds like me”, and then disappear.

We don't know who they were, what they were struggling with, what they wanted, or whether they were ready for support. The attention was real, and then it was gone.

| The biggest opportunity is what happens after someone stops scrolling.

Everything that follows is about capturing that moment — respectfully, and in a way that makes every future campaign smarter.

Serious social management, where every post has a job.

Run across both accounts in their roles, your content works to a clear purpose:

Recognition. Content that makes the right person think “that’s exactly what I’m experiencing” — symptoms people have normalised, feeling unwell despite “normal” results, hormonal change.

Education. What may be happening, the questions worth asking, and how Gina thinks about it — without ever diagnosing through content.

Authority. Your expertise, perspective and clinical philosophy — the depth that sets you apart.

Trust. You, your team, and what a consultation is actually like — so people feel safe taking a step.

Action. A logical next step. Increasingly not “book now”, but “take the educational assessment”.

That last step is where your content and the next stage meet.

The educational assessment: your biggest growth opportunity.

Instead of asking a stranger to book a consultation, we offer something more useful first.

A problem-aware message — a post or ad that names something real.



An educational assessment — a short, genuinely useful next step. Low commitment.



A personalised educational result — helpful guidance and routing. Never a diagnosis.



Consented follow-up — connected, where appropriate, into your clinic's existing communication.



A consultation opportunity — for the people who want the appropriate next step.

We lower the commitment at the start, while increasing what the clinic learns. *The assessment educates and guides. It never diagnoses, and never replaces a clinical consultation.*

Advertising becomes a way to reach more of the right people.

Not “pay Meta for more views”. Organic shows us what resonates; paid deliberately puts those proven messages in front of more relevant people; the assessment gives them a measurable next step; and follow-up turns interest into a patient journey.

THREE INITIAL HYPOTHESES TO TEST — NOT GUARANTEED WINNERS

HYPOTHESIS 1

“You've been told your results are normal. Why don't you feel normal?”

→ short educational assessment → guidance + next step → consented clinic journey.

HYPOTHESIS 2

Perimenopause and hormonal change, misunderstood.

→ symptom-awareness assessment → guidance → consented journey.

HYPOTHESIS 3

Longevity and biological-age curiosity (or men's health).

→ educational assessment → guidance → consented journey.

We start small, you control the budget, and we increase spend only when the evidence earns it.

Every response makes the next campaign smarter.

The assessment doesn't only capture interest. It quietly builds first-party insight — subject to consent, privacy and compliance. Over time we may learn which symptoms come up most, which problems bring people in, which questions recur, and which messages create the strongest interest.

That understanding feeds straight back in: we make **more content** around what people actually need, we test those themes in **paid** deliberately, and we improve the **information** people receive.

AUDIENCE → DATA → INSIGHT → BETTER CONTENT → SMARTER ACQUISITION



Your audience starts telling us what to create next.

A social manager posts; an ads manager buys traffic. This connects them into something that learns — which is why it's worth more than either on its own.

I've built this before, inside a health business.

This isn't an approach I'm proposing because it looks neat on paper. I've led marketing inside a health-related business where acquisition didn't end with “ad, then a sales call”. There was an intermediate assessment step that gave people a useful next action, captured their interest, helped qualify who was a fit, enabled structured follow-up, and created real data about what the audience was struggling with.

That experience is exactly why I'm recommending this shape for you. In a high-trust field, giving people the right next step, rather than forcing an immediate high-commitment decision, is what makes the difference.

Shared as experience with the method, not a promise of identical results.

Three workstreams, running in parallel.

We don't spend a month deciding what to think — the deep dive already did that.

Three tracks run at once and converge. Throughout, your part stays light.

1 • Organic growth — from week one

Apply the channel improvements we've already identified; set the two-account model; define content pillars; publish strategically; test hooks and formats; analyse June's best posts; optimise on response.

Content improvement begins in week one.

2 • The acquisition journey — early

Design Assessment V1 (educational, never diagnostic); build the entry journey and the qualification/routing; set up tracking; connect the agreed follow-up into your clinic's existing communication.

3 • Paid acquisition — once the entry journey is ready

Launch the initial test angles. Recommended minimum media budget **US\$1,000/month** — separate from my fee, paid to the platform, yours to control and adjust, never scaled automatically. Measure lead quality, build retargeting, optimise, and scale only what earns it.

By day 90, you'll have: a working social operating model, an active content engine, a live acquisition journey, real paid-media data, a qualification and assessment process, a clearer view of your acquisition economics, and the evidence for what to scale next. Not a promise of a set number of patients or a guaranteed return — a system producing real answers.

Built to grow with the clinic, not to be rebuilt every time it does.

- **Today:** you, your team, your current services.
- **Next:** repeatable, measured demand for those services.
- **Then:** the same foundation supporting new doctors as they join, new services and clinical focus areas, educational events, partnerships, and in time wider reach.

The point isn't to spend more on marketing. It's to build a growth pathway that doesn't have to be reinvented every time the clinic takes a step forward.

Your expertise, without another job.

You're a busy clinician, not a content team. This is designed around that.

I batch and repurpose so I'm not forever asking for more. **From you:** your expertise in focused sessions, clinical approval where it's needed, and the occasional review.

From me: the strategy, the content, the campaigns, the assessment, the measurement, and the constant refining.

| You bring the medicine. I run the growth.

Everything included, and what it's worth.

Most clinics buy these as separate services: a social media manager, an ads manager, an acquisition build, and someone to handle tracking. Here they're one partnership.

ONE - TIME

The Growth Build

\$2,500 · approximately £1,850

June breakout analysis · account architecture & roles · content strategy · paid-acquisition strategy · the educational assessment, built · the entry journey · tracking & measurement · reporting from day one.

MONTHLY · ALL PLATFORMS

The Growth Partnership

\$2,000 / month · approximately £1,480

Social strategy & management (Instagram + LinkedIn) · content, editing, repurposing, captions & creative · publishing, optimisation & testing · Meta campaign management & paid creative · the assessment & acquisition journey, refined · measurement, reporting & monthly strategy.

SEPARATE · YOU CONTROL IT

Advertising budget

recommended minimum \$1,000 / month · about £740

My recommended starting minimum — enough to run meaningful tests and begin learning. Separate from my fee, paid straight to the platforms, and entirely under your control to adjust; never scaled automatically.

Bought separately, your social management alone would usually take a full retainer, and paid campaign management another. Here they work as one. Fees are billed in US dollars; sterling figures are approximate, at today's exchange rate.

Let's map your first 90 days.

You've got the full picture of what I'm recommending. The call isn't to explain it again — it's to switch it on: confirm your capacity, align on priorities, finalise the scope, and agree a start date.

A focused 30 to 45 minutes, and we begin.

| Let's find a time this week.